PharmEasy Labs

Report for Shaik Shahin(23Y/F)

Tests asked Ek Sa 1428 01Hc Revise

Test date 17 Sep 2023 **Report status** Complete Report

REPORT VERIFIED VERIFIED BY DOCTOR

6 STEP quality control to ensure 100% report accuracy

- Qualified and trained technicians
- Temperature-controlled containers to store samples
- Strict quality checks on samples before processing
- Regular monitoring of lab analyzers by experts
- Assured machine inspection on a daily basis
- Verified reports by qualified pathologists

25+ Years of Trust & Experience | NABL Accredited Labs | 100+ Crore Samples Processed

PharmEasy Labs

Name : SHAIK SHAHIN(23Y/F)

Ref. By : SELF

ADDRESS :
NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS SECOND CROSS NEAR RICE MILL 524002 NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS SECOND CROSS NEAR RICE MILL 524002 RAMAMURTHY

Report Availability Summary

Full Report Available

Note : This is summary page. Please refer to the table below for the details

Test	Report Status
25-OH VITAMIN D (TOTAL)	Available
COMPLETE URINE ANALYSIS	Available
ERYTHROCYTE SEDIMENTATION RATE (ESR)	Available
FASTING BLOOD SUGAR(GLUCOSE)	Available
HbA1c	Available
HEMOGRAM - 6 PART (DIFF)	Available
IRON DEFICIENCY PROFILE	Available
IRON	Available
TOTAL IRON BINDING CAPACITY (TIBC)	Available
UNSAT.IRON-BINDING CAPACITY(UIBC)	Available
KIDPRO	Available
LIPID PROFILE	Available
LIVER FUNCTION TESTS	Available
T3-T4-TSH	Available
VITAMIN B-12	Available

Note : Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by : Thyrocare Technologies Ltd.

PharmEasy Labs

NAME	: SHAIK SHAHIN(23Y/F)	**HOME COLLECTION :**	
REF. BY	: SELF	NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS SECOND CROSS NEAR RICE MILL 524002 NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS SECOND	
TEST ASKED	: EK SA 1428 01HC REVISE		

TEST NAME	TECHNOLOGY	VALUE	UNITS
ERYTHROCYTE SEDIMENTATION RATE (ESR)	**WESTERGREN**	**83**	**mm / hr**

Bio. Ref. Interval. :-

Male : 0-15
Female : 0-20

Please correlate with clinical conditions.

Method:- WESTERGREN

Sample Collected on (SCT)	: 17 Sep 2023 08:46
Sample Received on (SRT)	: 17 Sep 2023 23:39
Report Released on (RRT)	: 18 Sep 2023 01:52
Sample Type	: EDTA
Labcode	: 1709101919/DG126
Barcode	: BD089296

Dr Anusha B, MD (Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

PharmEasy Labs

NAME : SHAIK SHAHIN(23Y/F)
REF. BY : SELF
TEST ASKED : EK SA 1428 01HC REVISE

HOME COLLECTION :
NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS SECOND CROSS NEAR RICE MILL 524002 NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS SECOND CROSS NEAR RICE MILL 524002

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c - (HPLC)			
	H.P.L.C	5.1	%

Bio. Ref. Interval. :

Bio. Ref. Interval.: As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	100	mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

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TEST ASKED : EK SA 1428 01HC REVISE

HOME COLLECTION :
NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS SECOND CROSS NEAR RICE MILL 524002 NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS

TEST NAME	VALUE	UNITS	Bio. Ref. Interval.
TOTAL LEUCOCYTES COUNT (WBC)	7.41	X 10^3 / µL	4.0 - 10.0
NEUTROPHILS	60	%	40-80
LYMPHOCYTE	29.8	%	20-40
MONOCYTES	4	%	2-10
EOSINOPHILS	5.8	%	1-6
BASOPHILS	0.1	%	0-2
IMMATURE GRANULOCYTE PERCENTAGE(IG%)	0.3	%	0.0-0.4
NEUTROPHILS - ABSOLUTE COUNT	4.44	X 10^3 / µL	2.0-7.0
LYMPHOCYTES - ABSOLUTE COUNT	2.21	X 10^3 / µL	1.0-3.0
MONOCYTES - ABSOLUTE COUNT	0.3	X 10^3 / µL	0.2 - 1.0
BASOPHILS - ABSOLUTE COUNT	**0.01**	**X 10^3 / µL**	**0.02 - 0.1**
EOSINOPHILS - ABSOLUTE COUNT	0.43	X 10^3 / µL	0.02 - 0.5
IMMATURE GRANULOCYTES(IG)	0.02	X 10^3 / µL	0.0-0.3
TOTAL RBC	4.38	X 10^6/µL	3.8-4.8
NUCLEATED RED BLOOD CELLS	0.01	X 10^3 / µL	0.0-0.5
NUCLEATED RED BLOOD CELLS %	0.01	%	0.0-5.0
HEMOGLOBIN	13.3	g/dL	12.0-15.0
HEMATOCRIT(PCV)	40.8	%	36.0-46.0
MEAN CORPUSCULAR VOLUME(MCV)	93.2	fL	83.0-101.0
MEAN CORPUSCULAR HEMOGLOBIN(MCH)	30.4	pq	27.0-32.0
MEAN CORP.HEMO.CONC(MCHC)	32.6	g/dL	31.5-34.5
RED CELL DISTRIBUTION WIDTH - SD(RDW-SD)	**48.4**	**fL**	**39.0-46.0**
RED CELL DISTRIBUTION WIDTH (RDW-CV)	**14.1**	**%**	**11.6-14.0**
PLATELET DISTRIBUTION WIDTH(PDW)	13.5	fL	9.6-15.2
MEAN PLATELET VOLUME(MPV)	11.1	fL	6.5-12
PLATELET COUNT	286	X 10^3 / µL	150-410
PLATELET TO LARGE CELL RATIO(PLCR)	34	%	19.7-42.4
PLATELETCRIT(PCT)	0.32	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

Please Correlate with clinical conditions.
Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)
(This device performs hematology analyses according to the Hydrodynamic Focussing (DC method), Flow Cytometry Method (using a semiconductor laser), and SLS- hemoglobin method)

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NAME : SHAIK SHAHIN(23Y/F)
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TEST ASKED : EK SA 1428 01HC REVISE

HOME COLLECTION :
NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS SECOND CROSS NEAR RICE MILL 524002 NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS SECOND

TEST NAME	OBSERVATION	UNITS	Bio. Ref. Interval.
Complete Urinogram			
Physical Examination			
VOLUME	3	mL	-
COLOUR	PALE YELLOW	-	Pale Yellow
APPEARANCE	CLEAR	-	Clear
SPECIFIC GRAVITY	1.01	-	1.003-1.030
PH	6.5	-	5-8
Chemical Examination			
URINARY PROTEIN	ABSENT	mg/dL	Absent
URINARY GLUCOSE	ABSENT	mg/dL	Absent
URINE KETONE	ABSENT	mg/dL	Absent
URINARY BILIRUBIN	ABSENT	mg/dL	Absent
UROBILINOGEN	Normal	mg/dL	<=0.2
BILE SALT	ABSENT	-	Absent
BILE PIGMENT	ABSENT	-	Absent
URINE BLOOD	ABSENT	-	Absent
NITRITE	ABSENT	-	Absent
MICROALBUMIN	10	mg/L	< 30
Microscopic Examination			
MUCUS	ABSENT	-	Absent
RED BLOOD CELLS	1	cells/HPF	0-5
URINARY LEUCOCYTES (PUS CELLS)	ABSENT	cells/HPF	0-5
EPITHELIAL CELLS	**7**	**cells/HPF**	**0-5**
CASTS	ABSENT	-	Absent
CRYSTALS	ABSENT	-	Absent
BACTERIA	ABSENT	-	Absent
YEAST	ABSENT	-	Absent
PARASITE	ABSENT	-	Absent

Method : Fully Automated DIRUI H-100 Urinalysis Dipstick Method, Microscopy

Sample Collected on (SCT) : 17 Sep 2023 08:46
Sample Received on (SRT) : 17 Sep 2023 23:36
Report Released on (RRT) : 18 Sep 2023 01:34
Sample Type : URINE
Labcode : 1709101813/DG126
Barcode : BJ640546

Dr Anusha B, MD (Path)

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Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

PharmEasy Labs
Registered Address: DOCON TECHNOLOGIES PRIVATE LIMITED, Office No 208, 209 & 210 Second floor, A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka, Andheri (East), Mumbai - 400072
Lab Address: Seshans Complex, No.140, Anna Salai, Saidapet, Chennai
7022 000 900

PharmEasy Labs

NAME	: SHAIK SHAHIN(23Y/F)	**HOME COLLECTION :**	
REF. BY	: SELF	NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS SECOND CROSS NEAR RICE MILL 524002 NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS SECOND	
TEST ASKED	: EK SA 1428 01HC REVISE		

TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	88.9	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

Please correlate with clinical conditions.

Method:- GOD-PAP METHOD

Sample Collected on (SCT)	: 17 Sep 2023 08:46
Sample Received on (SRT)	: 17 Sep 2023 23:39
Report Released on (RRT)	: 18 Sep 2023 02:13
Sample Type	: FLUORIDE
Labcode	: 1709101905/DG126
Barcode	: BJ754796

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TEST ASKED	: EK SA 1428 01HC REVISE		

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	**C.L.I.A**	**14.95**	**ng/mL**

Bio. Ref. Interval. :-

DEFICIENCY : <20 ng/ml || INSUFFICIENCY : 20-<30 ng/ml
SUFFICIENCY : 30-100 ng/ml || TOXICITY : >100 ng/ml

Clinical Significance:

Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health. Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome. Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml.

Kit Validation Reference: Holick MF. Vitamin D Deficiency. N Engl J Med. 2007;357:266–81.

Please correlate with clinical conditions.

Method:- Fully Automated Chemi Luminescent Immuno Assay

Sample Collected on (SCT)	: 17 Sep 2023 08:46
Sample Received on (SRT)	: 17 Sep 2023 23:36
Report Released on (RRT)	: 18 Sep 2023 02:28
Sample Type	: SERUM
Labcode	: 1709101811/DG126
Barcode	: BK134006

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TEST ASKED	: EK SA 1428 01HC REVISE		

TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12	C.L.I.A	266	pg/mL

Bio. Ref. Interval. :-

Normal : 211 - 911 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):5.0%, Inter assay (%CV):9.2 %;Sensitivity:45 pg/ml

Kit Validation reference:
Chen IW, Sperling MI, Heminger LA. Vitamin B12. In: Pesce AJ, Kaplan LA, eds. Methods in Clinical Chemistry. St. Louis: CV Mosby; 1987:569–73.

Please correlate with clinical conditions.

Method:- COMPETITIVE CHEMI LUMINESCENT IMMUNO ASSAY

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NAME : SHAIK SHAHIN(23Y/F)
REF. BY : SELF
TEST ASKED : EK SA 1428 01HC REVISE

HOME COLLECTION :
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TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON	PHOTOMETRY	92	µg/dL
Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170 **Method :** Ferrozine method without deproteinization			
TOTAL IRON BINDING CAPACITY (TIBC)	PHOTOMETRY	281	µg/dL
Bio. Ref. Interval. : Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl **Method :** Spectrophotometric Assay			
% TRANSFERRIN SATURATION	CALCULATED	33	%
Bio. Ref. Interval. : 13 - 45 **Method :** Derived from IRON and TIBC values			
UNSAT.IRON-BINDING CAPACITY(UIBC)	PHOTOMETRY	189.12	µg/dL
Bio. Ref. Interval. : 162 - 368 **Method :** SPECTROPHOTOMETRIC ASSAY			

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	PHOTOMETRY	169	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	48	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**119**	**mg/dL**	**< 100**
TRIGLYCERIDES	PHOTOMETRY	75	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	3.5	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	1.56	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	2.5	Ratio	1.5-3.5
HDL / LDL RATIO	CALCULATED	0.4	Ratio	> 0.40
NON-HDL CHOLESTEROL	CALCULATED	121	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	14.94	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
ALKALINE PHOSPHATASE	PHOTOMETRY	68.99	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.83	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.2	mg/dL	< 0.3
BILIRUBIN (INDIRECT)	CALCULATED	0.63	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	25	U/L	< 38
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	15.9	U/L	< 31
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	15.9	U/L	< 34
SGOT / SGPT RATIO	CALCULATED	1	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	7.73	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.34	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	3.39	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.28	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Vanadate Oxidation
BILD - Vanadate Oxidation
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	12.82	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.57	mg/dL	0.55-1.02
BUN / SR.CREATININE RATIO	CALCULATED	22.49	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	27.43	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	48.13	Ratio	< 52
CALCIUM	PHOTOMETRY	9.35	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	3.7	mg/dL	3.2 - 6.1

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

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PharmEasy Labs
Registered Address: DOCON TECHNOLOGIES PRIVATE LIMITED, Office No 208, 209 & 210 Second floor, A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka, Andheri (East), Mumbai - 400072
Lab Address: Seshans Complex, No.140, Anna Salai, Saidapet, Chennai
7022 000 900

PharmEasy Labs

NAME : SHAIK SHAHIN(23Y/F)
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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	C.L.I.A	88	ng/dL	60-200
TOTAL THYROXINE (T4)	C.L.I.A	6	µg/dL	4.5-12
THYROID STIMULATING HORMONE (TSH)	**C.L.I.A**	**12.69**	**µIU/mL**	**0.3-5.5**

Comments : IF NOT ON DRUGS SUGGESTED FT3 & FT4 ESTIMATION

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3 - Competitive Chemi Luminescent Immuno Assay
T4 - Competitive Chemi Luminescent Immuno Assay
TSH - Sandwich Chemi Luminescent Immuno Assay

Pregnancy reference ranges for TSH/USTSH :

Trimester	T3 (ng/dl)	T4 (µg/dl)	TSH/USTSH (µIU/ml)
1st	83.9-196.6	4.4-11.5	0.1-2.5
2nd	86.1-217.4	4.9-12.2	0.2-3.0
3rd	79.9-186	5.1-13.2	0.3-3.5

References :

1. Carol Devilia, C I Parhon. First Trimester Pregnancy ranges for Serum TSH and Thyroid Tumor reclassified as Benign. Acta Endocrinol. 2016; 12(2) : 242 - 243
2. Kulhari K, Negi R, Kalra DK et al. Establishing Trimester specific Reference ranges for thyroid hormones in Indian women with normal pregnancy : New light through old window. Indian Journal of Contemporary medical research. 2019; 6(4)

Disclaimer :

Results should always be interpreted using the reference range provided by the laboratory that performed the test.
Different laboratories do tests using different technologies, methods and using different reagents which may cause difference In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports.
To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Sample Collected on (SCT) : 17 Sep 2023 08:46
Sample Received on (SRT) : 17 Sep 2023 23:36
Report Released on (RRT) : 18 Sep 2023 02:28
Sample Type : SERUM
Labcode : 1709101811/DG126
Barcode : BK134006

Dr Anusha B, MD (Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

PharmEasy Labs

NAME : SHAIK SHAHIN(23Y/F)
REF. BY : SELF
TEST ASKED : EK SA 1428 01HC REVISE

HOME COLLECTION :
NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS SECOND CROSS NEAR RICE MILL 524002 NELLORE MYPAD ROAD NELLORE 4TH MILE BRUNDHAVANAM GARDENS SECOND

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	131	mL/min/1.73 m2

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- CKD-EPI Creatinine Equation

~~ End of report ~~

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